



AI Playbook and Toolkit for Public Health Departments

All-Staff Safe AI Use and Approved Tools Awareness

INT 105

This module prepares all public health staff to use AI safely, responsibly, and within the boundaries established by their agency. It is written for staff who may not design or govern AI systems but may use approved tools for drafting, summarizing, organizing information, or supporting routine work. The module focuses on practical decision-making: what staff should do before using AI, what information should never be entered, how outputs should be reviewed, and when concerns should be escalated. Because this curriculum is not tailored to a specific health department, it cannot identify which tools are approved in a given agency. Instead, the module teaches learners to look for agency-approved tools, acceptable use policies, data classification rules, privacy requirements, and review pathways before using AI. The goal is to prevent informal or shadow AI use that could expose sensitive data, create inaccurate outputs, or undermine public trust.

Level	Introductory
Primary track	Shared Foundational Course
Appears in tracks	shared-foundational

Learning Objectives

- Explain the difference between approved AI use and informal or shadow AI use.
- Identify types of public health information that should not be entered into unapproved AI tools.
- Apply a basic safe-use checklist before using AI for a work task.
- Describe when AI outputs require human review, supervisor review, or escalation.
- Use agency-approved tool awareness as a safeguard without assuming a specific tool is approved nationally.

Module Overview

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Jurisdiction and Agency Policy Note

This national-level module is intended for training and planning purposes. It does not replace legal, privacy, procurement, civil rights, records, cybersecurity, Tribal consultation, labor relations, human resources, or agency policy review. Learners should confirm applicable requirements in their own jurisdiction and organization before implementing AI-supported workflows.

Why This Topic Matters for Public Health AI

AI is increasingly available through search tools, office software, analytics platforms, transcription tools, chatbots, and vendor products. Staff may use these tools before the organization has completed review or before staff understand the risks. Public health agencies hold sensitive information about individuals, communities, outbreaks, services, partners, and operations, so even routine experimentation can create privacy, security, legal, equity, and trust concerns.

Safe use is a workforce competency, not only a technical issue. Every staff member needs enough AI literacy to recognize when a tool is appropriate, when a tool is not approved, when data are too sensitive, and when an output needs review. A shared baseline also helps supervisors, programs, IT, privacy, legal, communications, and governance bodies manage AI adoption consistently.

Public Health Example

A staff member wants to use an AI chatbot to summarize notes from a meeting about a community vaccination event. The notes include names of community partners, concerns raised by residents, operational challenges, and possible locations for future clinics. Even if no medical record is included, the notes may contain sensitive partner information, community trust concerns, and non-public planning details.

Before using AI, the staff member should check whether the tool is approved for meeting note summarization and whether the notes must be edited or generalized first. If no approved tool is available, the staff member should not paste the notes into a public AI platform. A safer approach may be to create a generalized summary manually or use

an approved internal tool that meets agency data protection requirements.

Technical, Programmatic, and Operational Deep Dive

Safe AI use begins with the use case, the tool, the data, and the intended output. Staff should ask whether the task is appropriate for AI, whether the tool is approved for that task, whether the information can be entered, and whether the output will be reviewed before use. These questions should be asked before experimentation begins, not after a draft, summary, or recommendation has already been generated.

Public health staff should understand the difference between low-risk productivity support and higher-risk operational or decision-support uses. Drafting a meeting agenda from non-sensitive notes may be low risk if the tool is approved, but summarizing identifiable case records, drafting public health guidance, prioritizing outreach lists, or recommending program decisions requires additional review. The same AI tool can be low risk in one context and high risk in another.

Supervisors and program managers should create clear expectations for staff. Staff should know where to find approved tool lists, who to ask when unsure, how to report an AI-related concern, and what types of AI use are prohibited. Without this clarity, staff may rely on personal judgment, which can create inconsistent and unsafe practices across the agency.

Risks, Failure Modes, and Guardrails

A common failure mode is shadow AI use, where staff use public or unapproved tools because they are convenient or because no approved alternative has been communicated. Shadow use can expose sensitive information, create records that the agency cannot audit, or produce outputs that are used without proper review. Agencies should make approved tool guidance easy to find and should provide safe alternatives when AI use is allowed.

Another risk is overtrusting AI output because it sounds confident, polished, or authoritative. AI can produce inaccurate facts, incomplete summaries, fabricated citations, outdated guidance, or tone that is inappropriate for public health communication. Staff should treat AI output as draft material and verify it against approved sources, subject matter experts, and agency policy.

Recommended guardrails include no-sensitive-data rules, approved tool lists, human review expectations, escalation pathways, and documentation requirements for higher-risk uses. Staff should be trained to pause when a task involves identifiable information, public-facing content, vulnerable populations, consequential decisions, or non-public operational details. When in doubt, the safest action is to ask before using AI.

Application to AI-Supported Workflows

In an AI-supported workflow, safe use rules should be built into the process rather than left to individual memory. Workflow instructions should state which tool is approved, what data may be used, what data are prohibited, who reviews the output, and what to do when the AI output is wrong or concerning. This makes responsible AI use repeatable and easier to supervise.

Staff should also understand that AI use may create records or drafts that need to be handled according to agency policy. A generated summary, prompt, output, or transcript may contain sensitive information or become part of a decision record. Agencies should clarify whether and how AI prompts, outputs, and review notes should be saved.

Reflection Questions

What types of information do you handle that should never be entered into an unapproved AI tool?

Where would you find your agency's approved tool list or acceptable use policy?

What AI-supported tasks in your role would require supervisor or governance review before use?

Practical Exercise

Choose one routine task from your own work or a realistic public health role, such as drafting a meeting summary, creating a training outline, summarizing public comments, or organizing a list of outreach ideas. Complete a safe AI use checklist for the task before deciding whether AI should be used. Do not use real sensitive data; use fictional, de-identified, or generalized information.

Your checklist should identify the task, the proposed AI use, the data involved, whether the tool would need to be agency-approved, what information must be excluded, who should review the output, and what concerns would require escalation. The goal is not to prove that AI should be used, but to show that you can make a safe and policy-aware decision.

Example: A health educator wants to use AI to draft an outline for a diabetes prevention workshop. The safe-use checklist notes that no participant names, referral data, or clinic records will be entered; the staff member will use only approved public guidance and agency-approved messages; the output will be reviewed for health literacy and cultural appropriateness; and the educator will confirm whether the AI tool is approved for drafting educational materials before use.

Expected Artifact or Evidence

Completed Safe AI Use Checklist

Brief note identifying whether the use is appropriate, requires revision, or requires supervisor/governance review

Knowledge Check

- 1. What is the most important first step after completing the All-Staff Safe AI Use and Approved Tools Awareness module?
- 2. Which practice best supports responsible AI use in public health?
- 3. Which statement best reflects the risk or guardrail for this module: A common failure mode is shadow AI use, where staff use public or unapproved tools because they are convenient or because no approved alternative has been communicated. Shadow use can expose sensitive information, create records that the agency cannot audit, or produce outputs that are used without proper review. Agencies should make approved tool guidance easy to find and should provide safe alternatives when AI use is allowed.
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- 4. When should a staff member escalate an AI-related concern?
- 5. What counts as stronger completion evidence for a training module?

References and Resources

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